

Instructions for Administration:

4 Months Old

Note Ensure to wash your hands before handling the child and to handle the child gently.

Definitions

Plane: In reference to items requiring holding a body part on the same plane as another, this refers to being level to.

Prone Position: The prone position refers to lying horizontally with the face and torso facing down. The infant lies with knees drawn up under the abdomen with pelvis high. Head is turned to one side.

Supine Position: The supine position refers to lying horizontally with the face and torso facing up.

Ventral Suspension: The baby is placed in prone position and suspended in the air by a hand placed under the chest. The head hangs down with some flexion of elbows and knees.

1.1 Gross Motor

Notes If the child does not perform the gross motor item for their age, but performs a more advanced gross motor item, then the item specific for their age is considered normal.

The first step towards the development of locomotion is the development of head control, that is, the ability to support the head in all positions of the body. The steps in this are observed in three situations: in ventral suspension, in prone position, and in supine position.

1.1.16	In prone position, holds chin and shoulders off surface	Age: 3 - 4 months
Materials	Padded Surface	
Instructions	The infant is placed in prone position with the head in the midline, and the movements of the head are noted for about 30 seconds. 	
Response	Holds chin and shoulders off surface so that the plane of the face is 45-90 degrees from the surface for 30 seconds.	
Grading	Present (30 seconds) For a few seconds or Lifts only chin, but not shoulders Lifts chin momentarily No attempt	

1.1.17	In sitting position, holds head up well consistently	Age: 4 - 5 months
Materials	Padded Surface	
Instructions	Sit the infant on a padded surface on the floor and keep their position steady. Provide trunk support by holding their forearms. 	
Response	Holds head up well, and constantly for at least two minutes.	
Grading	Present < 2 minutes Few seconds Poor Attempt	

1.2 Fine Motor Items

Note If the child does not perform the fine motor item for their age, but performs a more advanced fine motor item, then the item specific for their age is considered normal.

1.2.3	Hand posture	Age: 3 - 5 months
Materials	None	
Instructions	Observe the infant when awake and alert, and at rest. Note the posture of the hands. 	
Response	Hands should be open most of the time.	
Grading	Present Lightly fisted Semi-clenched Tightly clenched	

1.2.4	Holds objects placed in hands	Age: 3 - 4 months
Materials	Rattle, Padded Surface	
Instructions	Sit the infant in the caregiver's lap or place in supine position on a padded surface. Place a rattle in the infant's hand. Test both hands separately. 	
Response	Observe if the infant retains the object placed in the hand for several moments.	
Grading	Present Holds Momentarily Attempts No grasp	

1.2.5	Holds hands together in the middle	Age: 4 months
Materials	Padded Surface	
Instructions	Sit the infant in the caregiver's lap or place in supine position on a padded surface 	
Response	Observe if both the hands come together in the midline.	
Grading	Present With difficulty Attempts No attempt	

1.3 Vision Items

1.3.8	Fixates and follows 6cm spinning red ball	Age: 3 - 4 months
Materials	Red Ball on String If no response to Red Ball on String: Red Woolly Ball, Compact Mirror, Torch	
Instructions	If good head control has not yet developed, position the infant lying back, or up on a shoulder of the caregiver. As trunk control develops sit the infant supported upright in the caregiver's lap. Hold a spinning 6cm red ball suspended on a string about 25 cm from the infant's face to induce visual fixation. Then, move the ball slowly and horizontally on each side to induce full abduction, then vertically up and down after moving it back in front of the eyes. If no response, test by spinning the red woolly ball or reflecting object at the same distance; or a glowing torch beam at closer distance.	
Response	Fixates and follows the spinning 6cm Red ball on String	
Grading	Present Fixes/follows the 12.5cm spinning red woolly ball Fixes/follows reflecting object Fixes/follows light source only	



1.3.9	Watches moving adult from 3 metres	Age: 3 - 11 months
Materials	None	
Instructions	Stand in front of the infant at a distance of three metres and draw their attention. Move along an arc around the visual field of the infant and observe if the eyes move to track the moving adult. If no response, keep coming closer and observe the distance at which the infant's visual attention is engaged.	
Response	Infant watches moving adult from 3 metres.	
Grading	Present From 2m From 1m From <1m	

1.4 Hearing Items

Note Soft whisper - 10 to 25 db; conversational speech - 25 to 55 db; crying baby - 55 to 65 db; rattle - 60 to 80db.

1.4.7	Response to rattle	Age: 4 - 6 months
1.4.8	Response to Voice	Age: 4 - 6 months
Materials	Rattle, Assistant	
Instructions	Sit the infant on the caregiver's lap with head in the midline. The auditory stimulus is: (a) presented at the ear level, (b) about 45 cm from the ear, (c) out of the line of vision Shake the rattle gently. Test the right ear, then the left ear. Call the infant in a soft. Test the right ear, then the left ear.  	
Response	The infant turns head towards the side from which the sound comes without actual location of the sound source / shifts eye / alerts / changes facial expression. The best response to any one of the two sound sources is taken as normal.	
Grading	Present Questionable To loud rattle/voice only No response	

1.5 Expressive Language Item

Note This domain relies primarily on observation of infant vocalisations during the session:

- The examiner has to be vigilant about recording all infant vocalisations during the entire assessment so that none are missed.
- Interact and speak with the child to encourage vocalisations.
- If no vocalisations are made throughout the assessment, ask the caregiver if this is typical as it is not uncommon for young children to behave differently in new and unfamiliar situations. Document specific examples of vocalisations reported by the caregiver on the record form.

1.5.3	Tuneful vowel sounds e.g, “aa”, “uu”, “erghoo”	Age: 4 months
Materials	None	
Instructions	These are the first vowel sounds which are part of early pre-language speech development.	
Response	Note the frequency with which these sounds are being produced.	
Grading	Present Occasional Seldom No Sounds	

1.5.4	Chuckling, laughing	Age: 4 months
Materials	None	
Instructions	The infant will laugh loudly.	
Response	Note the frequency with which the infant chuckles and/or laughs.	
Grading	Present Occasional Seldom No Sounds	

1.6 Cognition Item

1.6.10	Looks at the rattle they are holding and plays with it	Age: 4 - 5 months
Materials	Rattle	
Instructions	Sit the infant upright on the caregiver's lap. Hold the rattle about 20-25cm from the infant. Shake it gently to attract attention and encourage the infant to reach. If the infant does not reach for the rattle, place it in their hands so that they play with it.	
Response	Observe if the infant looks at the rattle and plays with it, e.g., chewing or shaking, for several moments.	
Grading	Present Momentarily Looking present but no playing No looking but playing OR No looking or playing	

1.7 Behaviour Items

Note It is essential the examiner scores the Behaviour items based on their observations and caregiver report regarding sleep. Observe and record the child's behaviour according to the activities listed under this domain.

1.7.6	Sleep/awake rhythm	Age: 1 - 11 months
Materials	None	
Instructions	Ask the caregiver about the infant's sleep patterns.	
Response	Score according to caregiver's recall. Over the first 3 months of life the total amount of sleep declines little, but the cycles are longer so that, by the age of 3 months, 70% of infants are sleeping right through the night. By 6 months, 80% of infants sleep through the night, but there are still 10% waking every night by the age of 1 year. By this age, the great majority of infants have established a stable pattern of sleep and wakefulness with a long sleep at night and a nap in the morning and afternoon. Mild, moderate and severe grades of response are: occasional disturbance, considerable disturbance, and constant problem. Either the infant is sleeping too much or is awake for longer periods of time.	
Grading	Normal sleep patterns Occasional disturbance Considerable disturbance Constant problems	

1.7.7	Displays positive affect most of the time	Age: 1 - 11 months
Materials	None	
Instructions	Note the infant's affect.	
Response	The grading is done according to the degree of negative affect displayed during the assessment. Positive affect comprises of smiling, laughing, sound making that is perceived as the expression of excitement, happiness or pleasure. Negative affect includes fussing, pouting, whining, crying, vocal or physical expressions of anger.	
Grading	Displays positive affect Sometimes negative Mostly negative Constantly negative	

1.7.8	Displays normal periods of quietness	Age: 3 - 11 months
Materials	None	
Instructions	This item tests the child's quietness. Few children are so quiet that they are very unresponsive.	
Response	The infant is quiet, responsive, not restless.	
Grading	Displays normal periods of quietness Placid/Quiet Unusually Quiet Extremely Quiet	

1.7.9	Displays responsiveness to surroundings	Age: 3 - 11 months
Materials	None	
Instructions	Observe if infant engages in visual, auditory, oral, or physical exploration. For example, trying to touch or hold objects.	
Response	Degree of responsiveness is noted. A curiosity towards new stimuli within the environment is normal.	
Grading	Present Most of the time Infrequent Never	

1.7.10	Good eye contact	Age: 3 - 11 months
Materials	None	
Instructions	Note the frequency of eye contact the infant makes with the examiner.	
Response	The infant should be making eye contact with persons interacting with them.	
Grading	Present Most of the time Infrequent Never	

1.8 Seizure Item

1.8.3	Seizures (provoked or unprovoked)	Age: 3 - 5 months
Materials	None	
Instructions	Ask caregiver or check medical records to determine whether infant has ever experienced a seizure. Seizures are manifestations of neuronal injury due to excessive electrical activity in the brain. Any disturbance in the brain may cause seizures, which may be single or repetitive, and involves a sudden change in behaviour.	
Response	Consider typical if the infant has not had a provoked or unprovoked seizure.	
Grading	No seizures (provoked or unprovoked) ≥ Once, but none in the past month ≥ Once, also in the past month > Once, also in the past week	